

ICD-0-3 Carcinoma, infiltrating lobular 8520/3
Site: breast, NOS C50.9 1127/11 per

Name: [REDACTED] Age/Sex: /F Location:
Acct#: [REDACTED] Unit#: [REDACTED] Status: DIS IN Room/Bed:
Reg: [REDACTED] Disch: [REDACTED] Att Dr: [REDACTED]

Specimen: Received: Status: Req#:
Spec Type: SURGICAL P Subm Dr:

PREOPERATIVE DIAGNOSIS

LEFT BREAST CANCER INVASIVE

OPERATION PERFORMED

DATE:

DOCTOR(S):

PROCEDURE: BILATERAL SIMPLE MASTECTOMY-SENTINEL NODE BX

ISSUE RETURNED

- A. LT SENTINEL NODE #2 - FS 0/1
B. LT SENTINEL NODE #1 0/1
C. LT SIMPLE MASTECTOMY
D. RT SIMPLE MASTECTOMY
E. RT SENTINEL NODE #1 0/1
F. RT SENTINEL NODE #2 0/1
H. POSSIBLE LT SENTINEL NODE 90
G. LT SENTINEL NODE #3 90

UUID:377F7050-F900-4345-A326-568F000508C3
TCGA-A2-A0T6-01A-PR

Redacted



POST-OPERATIVE DIAGNOSIS

A. BENIGN

GROSS DESCRIPTION

PART A RECEIVED FRESH LABELED [REDACTED] LEFT SECOND SENTINEL NODE PALPABLE, CONSISTS OF AN IRREGULAR PORTION OF YELLOW-RED FATTY TISSUE MEASURING 2.5 X 1.0 X 1.0 CM. SECTIONING REVEALS 2 LYMPH NODES, THE LARGER 0.9 CM IN DIAMETER DEMONSTRATES A RIBBON CLIP. THE SECOND IS 0.5 CM. SECTION OF THE LARGER IS SUBMITTED AS PSA. THE REMAINDER OF THE SPECIMEN IS SUBMITTED LABELED A.

PART B RECEIVED FRESH LABELED [REDACTED] LEFT FIRST SENTINEL NODE HOT AND BLUE, IS A PORTION OF YELLOW-RED FATTY TISSUE MEASURING 2.2 X 1.5 X 1.0 CM. SECTIONING REVEALS GROSSLY UNREMARKABLE NODAL TISSUE WITH FOCAL BLUE DYE. ONE HALF IS SUBMITTED PER PROTOCOL, THE REMAINDER IS SUBMITTED LABELED B.

PART C RECEIVED FRESH LABELED [REDACTED] LEFT SIMPLE MASTECTOMY STITCH AT 12 O'CLOCK, IS A MASTECTOMY SPECIMEN WITH OVERALL DIMENSIONS OF 23.5 X 20.5 X 4.5 CM. THE NIPPLE IS UNREMARKABLE AND IS WITHIN AN IRREGULAR SHAPED SKIN EXCISION MEASURING 8.0 X 4.3 CM IN GREATEST

Patient: [REDACTED]

(Continued)

Specimen:

Received:

Status:

Req#:

Spec Type: SURGICAL P

Subm Dr:

GROSS DESCRIPTION

(Continued)

DIMENSIONS. A SUTURE DENOTES 12 O'CLOCK AND BLUE DISCOLORATION IS NOTED IN THE UPPER INNER QUADRANT. THE SUPERFICIAL ASPECT IS MARKED WITH BLUE INK, THE DEEP MARGIN IN BLACK INK. SECTIONING REVEALS DENSE PINK-TAN FIBROUS NODULE IN THE MIDPORTION OF THE SPECIMEN WHICH IS CENTRAL AND IN THE SUPERIOR ONE-HALF OF THE SPECIMEN TO INCLUDE BOTH THE OUTER AND INNER QUADRANTS. IT HAS AN IRREGULAR SHAPE WITH NO PERCEPTIBLE DIFFERENTIATION BETWEEN THE LOBULATED ARMS CREATING A 7.0 CM SPAN FROM MEDIAL TO LATERAL 3.5 CM FROM SUPERIOR TO INFERIOR AND UP TO 2.5 CM FROM ANTERIOR TO POSTERIOR. THIS EXTENDS TO WITHIN 0.7 CM OF THE DEEP MARGIN. THERE ARE SMALL PALPABLE NODULES SURROUNDING THIS. ONE OF THESE IN THE UPPER INNER QUADRANT MEASURES 0.5 CM AND IS 1.3 CM FROM THE DEEP MARGIN. GROSSLY THIS IS NOT IN CONTINUATION WITH THE LESION AND SEPARATED BY 1 CM. THE PERIMETER TISSUE IS BLAND YELLOW FATTY TISSUE. SECTIONS ARE SUBMITTED AS FOLLOWS: C1--NIPPLE AND SKIN (MIRROR IMAGE TO PROTOCOL), C2--THE FAR MEDIAL PORTION OF THE LARGE LESION IN THE UPPER INNER QUADRANT, C3--THE SMALL NODULE IN THE UPPER INNER QUADRANT, C4--TISSUE FROM LOWER INNER QUADRANT 2 CM INFERIOR TO C2, C5 THROUGH C7--A SUPERIOR TO INFERIOR CROSS-SECTION OF THE LESION IMMEDIATELY SUBAREOLAR, C8--LESION AND DEEP MARGIN (MIRROR IMAGE TO PROTOCOL SECTION), C9 AND C10--SECTIONS OF PALPABLE LESION FAR LATERAL, SUPERIOR, AND INFERIOR, C11--LESION AND SUPERFICIAL MARGIN, C12--THE SUPERFICIAL PORTION OF THE LESION AND SKIN CORRESPONDING TO C8, C13--LOWER OUTER QUADRANT 3 CM FROM TUMOR, C14--UPPER OUTER QUADRANT 3 CM FROM TUMOR, C15--TUMOR (MIRROR IMAGE TO PROTOCOL), C16--TISSUE 6 O'CLOCK.

PART D RECEIVED FRESH LABELED [REDACTED] RIGHT SIMPLE MASTECTOMY STITCH AT 12 O'CLOCK, IS A SIMPLE MASTECTOMY SPECIMEN MEASURING 16.0 X 15.5 X 4.0 CM. THE NIPPLE IS UNREMARKABLE WITHIN A 7.2 X 4.3 CM SKIN ELLIPSE. A SUTURE DENOTES 12 O'CLOCK. THE SUPERFICIAL ASPECT IS MARKED WITH BLUE INK, THE DEEP WITH BLACK. SECTIONING REVEALS A CENTRAL PORTION OF THE BREAST TO CONSIST OF PINK-TAN FIBROUS TISSUE WITH MULTIPLE FLUID-FILLED CYSTS. THIS IS SURROUNDED BY A PERIMETER OF BLAND YELLOW FATTY TISSUE WITH FINE FIBROUS BANDS. THE FIBROUS COMPONENT IS 50% OF THE SPECIMEN. ONE CYST IN THE 9 O'CLOCK AREA CONTAINS TAN PUTTY-LIKE MATERIAL (D3). THE SPECIMENS ARE SUBMITTED AS FOLLOWS: D1--NIPPLE, D2--9 O'CLOCK AREA (MIRROR IMAGE TO PROTOCOL BLOCK), D3--CYST IN 9 O'CLOCK AREA, D4--LOWER OUTER QUADRANT, D5--UPPER OUTER QUADRANT, D6--CENTRAL DEEP MARGIN, D7 AND D8--LOWER INNER QUADRANT, D9 AND D10--UPPER INNER QUADRANT.

PART E RECEIVED FRESH LABELED [REDACTED] RIGHT SENTINEL NODE BLUE NOT HOT, IS FATTY TISSUE MEASURING 2.8 X 2.5 X 1.0 CM. SECTIONING REVEALS A 0.8 CM IN DIAMETER RED-TAN NODE. HALF IS SUBMITTED FOR PROTOCOL, THE REMAINDER IS SUBMITTED LABELED E.

PART F RECEIVED FRESH LABELED [REDACTED] SECOND RIGHT SENTINEL NODE

Patient: [REDACTED]

(Continued)

Specimen:

Received:

Spec Type: SURGICAL P

Status:

Subm Dr: [REDACTED]

Req#:

GROSS DESCRIPTION

(Continued)

HOT BLUE, IS AN OVOID PORTION OF RED-TAN TISSUE MEASURING 2.5 X 1.3 X 1.0 CM, IS A 1.2 CM LYMPH NODE SHOWING BLUE DISCOLORATION. ONE-HALF IS SUBMITTED PER PROTOCOL, THE REMAINDER IS SUBMITTED LABELED F.

PART G RECEIVED FRESH LABELED [REDACTED] LEFT FOURTH SENTINEL NODE HOT ONLY, IS YELLOW FATTY TISSUE MEASURING 2.3 X 1.5 X 0.8 CM. IT REVEALED A 1 CM GROSSLY UNREMARKABLE LYMPH NODE. ONE HALF IS SUBMITTED PER PROTOCOL, THE REMAINDER IS SUBMITTED LABELED G.

PART H RECEIVED FRESH LABELED [REDACTED] POSSIBLE LEFT SENTINEL NODE HOT NOT BLUE, YELLOW-FATTY TISSUE. THIS NODAL TISSUE IS IDENTIFIED AND THE SPECIMEN IS SUBMITTED ENTIRELY LABELED H.

PATH PROCEDURES

PROCEDURES:

88307/8, IMMUNOPEROXIDAS/6, PATH FS, ABX X6, BBX X6, C BLK/16, D BLK/10, EBX X6, FBX X6, FROZ.SEC. A, GBX X6, HBX X6

FINAL DIAGNOSIS

PARTS A AND B LEFT SECOND AND FIRST AXILLARY SENTINEL LYMPH NODE BIOPSIES: LYMPH NODES WITH NO EVIDENCE OF METASTATIC DISEASE, SUPPORTED BY NEGATIVE CYTOKERATIN IMMUNOHISTOCHEMICAL STAINING.

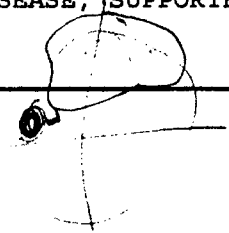
PART C LEFT SIMPLE MASTECTOMY: MULTICENTRIC IN SITU AND INFILTRATING LOBULAR CARCINOMA. THE TUMOR IS NUCLEAR GRADE II/III WITH A LOW MITOTIC INDEX. INVASIVE CARCINOMA SPANNED A GROSS DISTANCE OF 7.0 CM FROM MEDIAL TO LATERAL IN THE SUPERIOR CENTRAL PORTION OF THE SPECIMEN, SPANNING THE OUTER AND INNER QUADRANTS, WITH A DISTINCT AND SEPARATE 1.0-CM NODULE OF INVASIVE LOBULAR CARCINOMA IN THE UPPER OUTER QUADRANT OF THE BREAST. THE SUPERFICIAL AND DEEP MARGINS ARE FREE OF TUMOR BY AT LEAST 1 CM.

PART D RIGHT SIMPLE MASTECTOMY: FIBROCYSTIC CHANGES WITH FOCALLY MARKED NON-ATYPICAL INTRADUCTAL EPITHELIAL HYPERPLASIA PRESENT. NO EVIDENCE OF MALIGNANCY.

PARTS E AND F RIGHT FIRST AND SECOND AXILLARY SENTINEL LYMPH NODE BIOPSIES: LYMPH NODES WITH NO EVIDENCE OF METASTATIC DISEASE, SUPPORTED BY NEGATIVE CYTOKERATIN IMMUNOHISTOCHEMISTRY.

PART G LEFT FOURTH AXILLARY SENTINEL LYMPH NODE, BIOPSY: LYMPH NODE WITH NO EVIDENCE OF METASTATIC DISEASE, SUPPORTED BY NEGATIVE CYTOKERATIN IMMUNOHISTOCHEMICAL STAINING.

0/2 sub (L)
1cm apart - for
ILC - multicentric
upper mid
UOQ 1.0cm ILC
PCC
IDH-2
P 0/2
L 0/1



Patient: [REDACTED]

(Continued)

Specimen:

Received:

Status:

Req#:

Spec Type: SURGICAL P

Subm Dr:

FINAL DIAGNOSIS

(Continued)

PART H POSSIBLE LEFT AXILLARY LYMPH NODE, BIOPSY: ADIPOSE TISSUE WITH
NO LYMPH NODE PRESENT. p

Signed _____ (signature on file) _____

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	[Signature]	[Signature]
Qtrs Reviewed:	10/10	